Staffing, Scheduling and Shift Policy

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1. Purpose

This policy explains how staff are rostered, how safe staffing levels are defined, and how shortages are handled at CareOps General Hospital. Staffing is an operational resource, and low staff availability limits the number of beds that can be used safely even if physical beds are empty.

2. Shifts and handover

Nursing and support shifts are Morning 07:00 to 15:00, Evening 15:00 to 23:00 and Night 23:00 to 07:00. Handover between shifts happens at the bedside or in the unit and uses the SBAR format, which stands for Situation, Background, Assessment and Recommendation. SBAR is a standard communication tool used in healthcare. Each handover lasts up to 15 minutes. The outgoing staff member stays until handover is complete.

3. Target nurse-to-patient ratios

The hospital internal targets follow nurse staffing ratios commonly cited from the California hospital regulations. These are the maximum number of patients per nurse on a shift. Emergency Department: 1 nurse for 4 patients. ICU and other critical care: 1 nurse for 2 patients. Neonatal ICU: 1 nurse for 2 babies. General Medicine and General Surgery wards: 1 nurse for 5 patients. Cardiology ward with telemetry: 1 nurse for 4 patients. Pediatrics: 1 nurse for 4 patients. Active labour in Obstetrics: 1 nurse for 2 patients. Operating theatre: 1 circulating nurse for each patient under surgery. A ratio worse than the target is a patient safety risk and is reported to the Duty Manager.

4. Staff availability levels

Staff availability is the number of staff present divided by the number required by the roster. Adequate means 95 percent or more. Reduced means 85 to 94 percent and is reported to the Bed Manager. Low means 70 to 84 percent, is an Orange alert and may reduce the number of staffed beds. Critical means below 70 percent, is a Red alert and triggers the call-back procedure and the Level 3 review in the Overcrowding and Surge Capacity Plan.

5. Absence and call-in

A staff member who cannot attend a shift must inform the charge nurse at least 2 hours before the shift starts. The charge nurse informs the Duty Manager. The first cover option is the float pool of 30 nurses. The second option is voluntary overtime or an extra shift from off-duty staff. The third option is agency staff who have completed the hospital orientation. The fourth option is to reduce the number of staffed beds or to postpone elective work.

6. Working time limits

To protect patient safety and staff health, no employee works more than 12 consecutive hours. Overtime is limited to 16 hours per week per employee in normal conditions. In a declared Level 3 event the Medical Superintendent may approve a higher limit for a short period, but every employee must have at least 8 hours of rest before the next shift. Night shift staff may

not work more than 5 consecutive night shifts.

7. Doctor coverage

Each department has a consultant on duty during the day and a consultant on call at night who can reach the hospital within 30 minutes. Resident doctors are on 24 hour duty in the ED, ICU and wards under supervision. The ED has at least 2 doctors on duty at all times and at least 4 doctors during peak hours from 10:00 to 14:00 and from 18:00 to 22:00. The ICU always has a doctor in the unit.

8. Competency and training

All clinical staff complete Basic Life Support training every 2 years. ED and ICU staff complete Advanced Life Support training. New staff complete a 5 day orientation that covers infection control, fire safety, emergency codes, patient identification and data privacy. Float pool nurses are trained in at least 2 departments such as ED, ICU or medicine.

9. Monitoring staff wellbeing

Human Resources reviews overtime, sick leave and resignation trends every month. A department with overtime above the limit for 2 consecutive months receives an additional staffing review. Fatigue and burnout are treated as safety risks.